

ACA ADMIN AO3 POSITION HANDBOOK

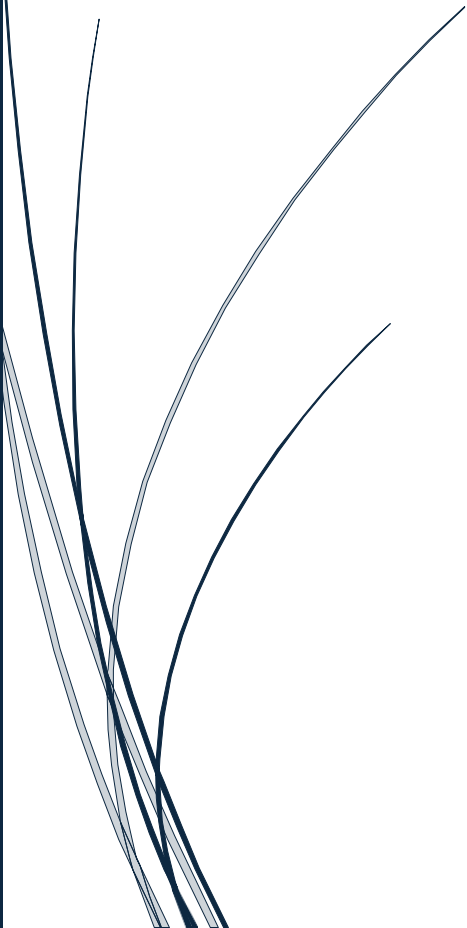




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ACA ADMIN A03 POSITION HANDBOOK

*"To care for those who once cared
for us is one of the highest honours."
- Tia Walker*



Metro South Aged Care Assessment Service
QUEENSLAND HEALTH

Contents

0 ACA Role Summary.....	3
1 ACA Admin Processes/Programs	4
1.1 Daily Process Overview	4
1.2 How to log onto MAC.....	5
1.3 How to log onto CHIMS	7
1.4 Accessing G Drive (Action Sheets).....	9
1.5 ACA Documentation – Maintaining privacy and confidentiality	10
1.6 MAC Basics	11
1.6.1 Find a client	11
1.6.2 Assessment referrals.....	11
1.6.3 Client Record	12
1.6.4 Review Requests	12
2 Appointments and bookings	14
2.1 Overview of referrals.....	14
2.2 Planning appointments	15
2.3 Calling clients.....	16
2.4 Home Visit Risk Assessment (HVRA).....	17
2.5 CHIMS process	18
2.5.1 Creating new CHIMS client	18
2.5.2 Creating Appointment.....	19
2.6 MAC process	21
2.6.1 Phone assessments	21
2.6.2 Clinic assessments	21
2.7 Booking interpreters	22
2.8 Cancellations and rescheduling appointments.....	23
2.8.1 Cancelling	23
2.8.2 Rescheduling.....	23
3 Letters and mail.....	24
3.1 Action sheets.....	24
3.1.1 Approval letter, non-approval letter and SPR letter.....	24
3.1.2 Support plan.....	24
3.1.3 Referral code letter.....	24
3.1.4 MAC - DPS Perm and Home Care	25

3.1.5 MAC – Carer Gateway Fact Sheet.....	25
3.1.6 MAC – Brochures	25
3.2 Mail.....	26
3.2.1 Surveys for mail.....	27
3.3 Emails.....	28
3.4 Fax	29
3.5 Steps to send out letters	30
3.5.1 Client correspondence.....	30
3.5.2 GP correspondence.....	30
3.5.3 Copies with consent	30
3.5.4 Hospital correspondence	30
3.6 Hospital Letters	31
3.7 Blank Action Sheet.....	32
4 Phones, emails and responding to enquiries.....	33
4.1 Phone and email process	33
4.2 Scenarios.....	34
4.2.1 Client enquires about wait time	34
4.2.2 Client wants priority assessment.....	34
4.2.3 Client/Supporter wants copy of paperwork or referral code.....	34
4.2.4 Client is returning a call to someone.....	34
4.2.5 Client is wanting to add a supporter.....	35
4.2.6 Other	35
4.3 Email responses	36
5 Miscellaneous tasks.....	37
5.1 Surveys	37
5.2 RiskMan	38
5.3 Ordering MAC brochures.....	39
5.4 Stocktake and WINC ordering	40
6 ACAT Process Template and Notes.....	42
7 Local Emergency Response Procedures.....	43

0 ACA Role Summary

The Government offers various funded Aged Care services to Elderly Australians in Queensland, with these primarily being the Support at Home program, Commonwealth Home Support Program (CHSP), Residential Permanent (nursing homes etc) and Residential Respite (temporary nursing home). Access to these programs and their funding are approved through Aged Care Assessments, in which clients will apply for a referral with My Aged Care (the national platform that hosts all aged care activity), after which an interview process by qualified delegates of the Government (our assessors) deems a client's needs and requirements, granting them access to the services.

Our job entails booking appointments for these Aged Care Assessments according to our referral waitlist, sending out paperwork after the assessment and answering any inquiries that may pop up throughout this process.

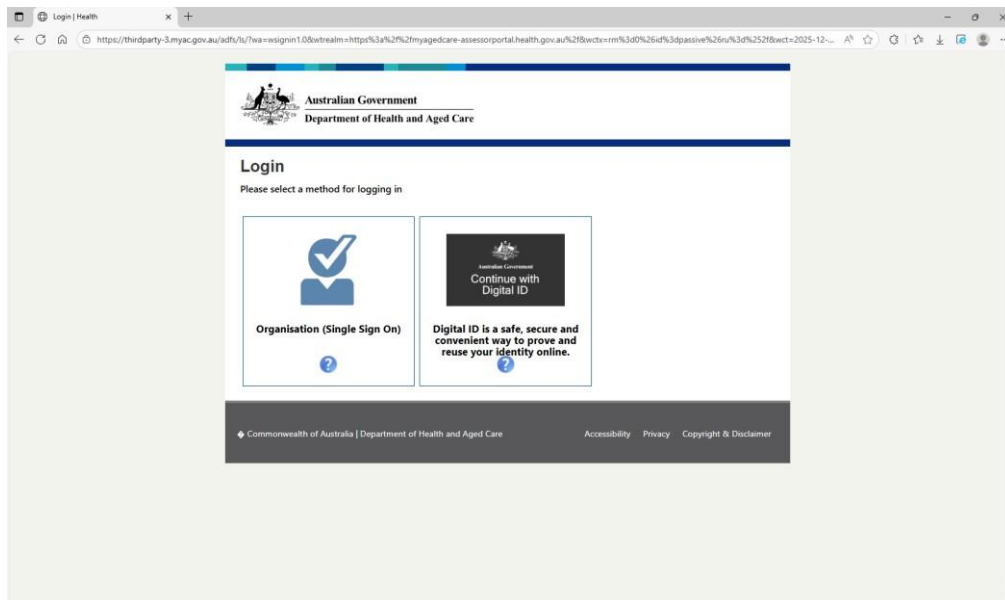
1 ACA Admin Processes/Programs

1.1 Daily Process Overview

- Sign on/login
- If first on phone:
 - Check phone messages and emails from overnight and follow up when time permits (see 4)
 - Generic email is: [REDACTED]
 - Phone number is: [REDACTED]
- Cancel clinic's/client appointments as directed by the clinical lead
- Check emails, and reply/follow up as necessary
- Conduct ACA Team Meeting in the morning:
 - Discuss phone allocations, daily tasks and priority areas
- Post/email Hospital letters as per action sheets found in the Hospital folder on the G drive (see 3)
- Post/email Community letters as per action sheets found on G drive of your relevant area (see 3)
- Check multiple view diary on CHIMS for available appointments for your designated area and make appointments as appropriate (assist others if required) (see 2)
- All outgoing mail to be downstairs by 1pm
- During allocated phone time:
 - Check for voicemails within your allocated time and follow up as necessary (see 4.1)
 - Check and action emails within your allocated time and follow up as necessary (see 4.1)
 - Keep record of these messages/emails in case of follow up in future
- Complete side-tasks during downtime or when not busy (see 5)

1.2 How to log onto MAC

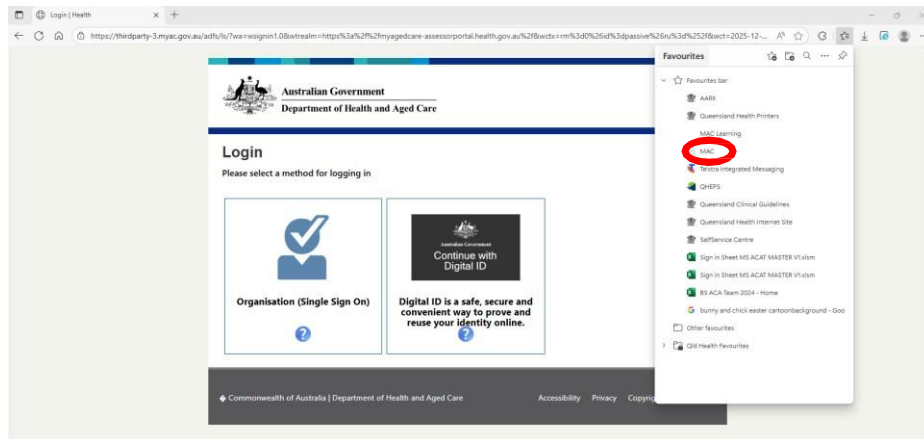
- Access provided by Clinical Director once training has been completed
- Open Chrome/Edge
- In search bar type:
 - <https://myagedcare-assessorportal.health.gov.au/>



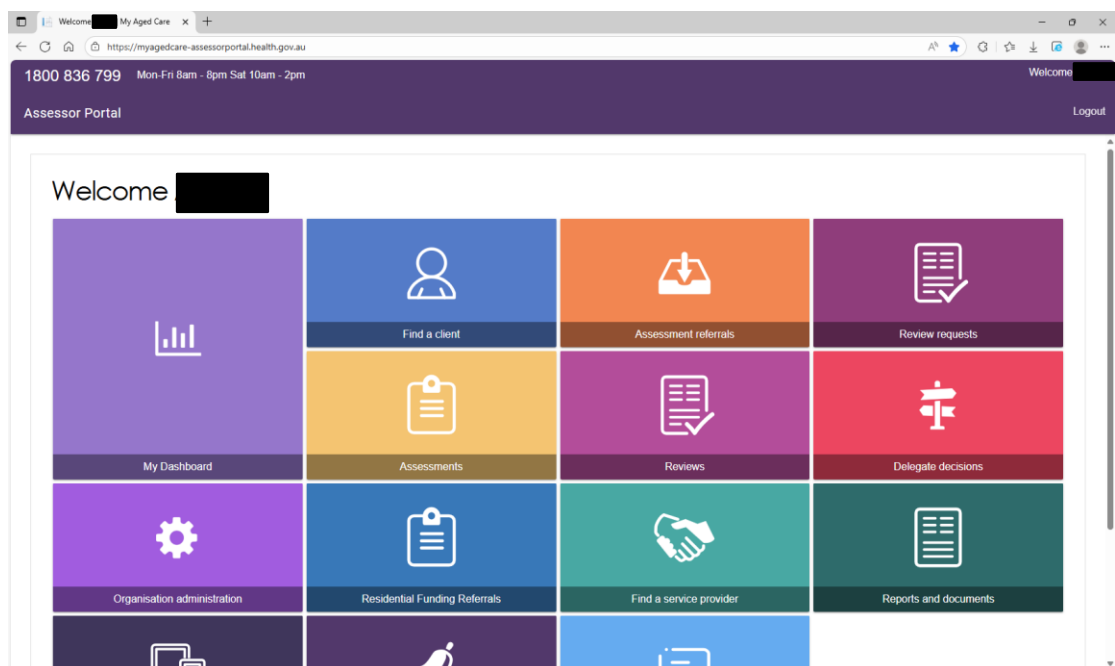
- Click on Organisation (Single Sign On)
- Follow these prompts for first time logon:
 - Type in Dept of Health Qld -> Continue
 - Type in your email and Novell password
 - Verification Code
- After logging on, save this page as a favourite for easy access



- Click the shooting star in the top right corner
- 
- Click the “star-plus” symbol (in blue) to add as a favourite, you can name it something relevant like “MAC”
 - Now you can access MAC via the favourites bar (see below)



- MAC looks like below:



- You are ready to start using MAC!

1.3 How to log onto CHIMS

- Obtain access from line manager



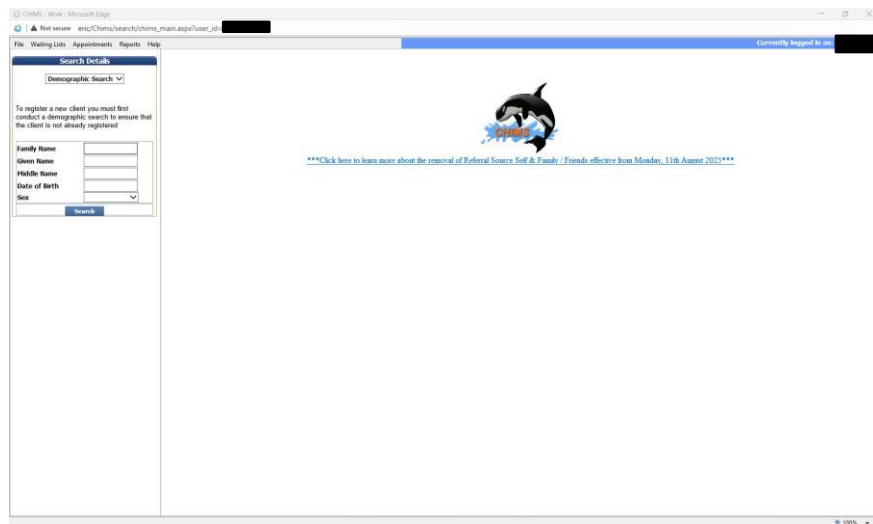
- Click on ERIC from Desktop

The ERIC login screen is displayed within a window titled "ERIC". It features the Queensland Government logo and the Metro South Health logo at the top. In the center is a large cartoon penguin with the word "ERIC" below it. To the right, there are input fields for "User Name:" and "Password:", followed by a red "ERIC Live Site" label and a blue "Login" button.

- Enter your username and password and press enter or click login, a new window will open. You can close the old window.



- Click on ERIC Clinical. Again, a new window will open, you can close the previous window.



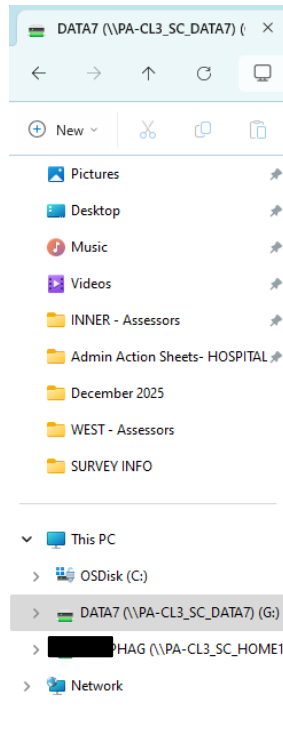
- Once you see this screen, you are in CHIMS. Click on Appointments, and then Multiple Diary in the dropdown to access the multiple diary view.

1.4 Accessing G Drive (Action Sheets)

- Obtain access from line manager



- Open File Explorer



- Click on DATA7 in the sidebar. 3 items
- Navigate to:
 - CommType
 - ACAT
 - Remote Assessment
 - For Community Letters:
 - Admin Action Sheets -COMMUNITY AXs
 - For Hospital Letters:
 - Admin Action Sheets- HOSPITAL AXs

1.5 ACA Documentation – Maintaining privacy and confidentiality

Procedure to follow when:

- Handling
- Posting
- Sorting
- Photocopying
- Compiling
- Uploading

All documentation relating to client information and resources that are available to Metro South Health clients.

Under no circumstances should documentation be distributed to clients via mail or in person unless a complete and thorough check is given to the documentation to ensure all pieces of paper are:

- Only the relevant client's information
- Only paperwork pertaining to the relevant master document copy e.g. Residential Care listing
- Are complete and in correct order

The checking of documents should include:

- Looking at 1st page and last page and checking each page in between
- Checking that the number of pages is all accounted for
- Checking the name is always for the same person
- Checking that the quality of the printed document is appropriate and legible
- Understanding the substance of the information you are handling and the priority in achieving 100% accuracy
- Having a mutual understanding of confidentiality and understanding and believing that the information is confidential

Photocopying of documents:

- Prior to photocopying of documentation please ensure there is nothing left in the tray prior to pressing the start button
- If there is a document left over from a previous user either – hand to staff member if know – place in the trays provided for pickup later
- Once your photocopying is completed check that no other copies are caught between your copies

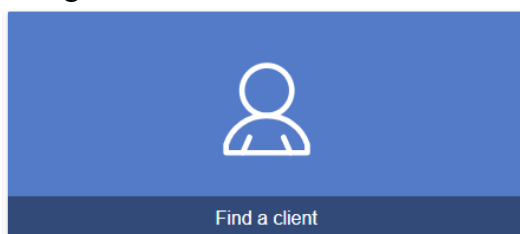
By protecting our business documents, it provides other benefits. Protecting our documents can create more efficient processes, a reduced risk of privacy breaches, and a reduction in the time and resources dedicated to detecting such breaches. On a professional level, if our data security is compromised, it can cause severe damage to our business, as well as a loss of privacy for your clients.

1.6 MAC Basics

1.6.1 Find a client

TO find a client record on MAC:

- Navigate to the *Find a client* section from the main menu or the upper navigation bar



- It will look like this:

The "Find a client" search interface. It features a search bar with fields for "Last name", "First name", and "Aged Care user ID". Below the search bar are buttons for "MODIFY FILTER OPTIONS", "CLEAR FILTERS", "SEARCH", and "CLEAR". To the right, there is a "Recently Viewed Persons" section with a placeholder image.

- You may search using the client's name or AC number. Other filters are available but either a name or AC number are still required. The tab on the right shows your most recently viewed clients for quick access.

1.6.2 Assessment referrals

Assessment referrals is a list where clients who are currently in our incoming or accepted list can be seen. It looks like this:

The "Incoming referrals" list interface. It shows a header with "Incoming referrals", "Accepted referrals", and "Referral history" tabs. Below the tabs is a "Filter by" section with a "Select all" checkbox. The main area displays a table with columns: Name, Aged care user ID, Locality, Assessment type, Date referred, Date due, Priority, and Select. The table contains 293 matching results, with the first 50 displayed. The table is currently empty, showing only the header and a large black redaction box.

- You can navigate the various lists by clicking the relevant tab at the top. In the upper right, the current displayed outlet can be changed. The default outlet should be *Dept of Health QLD – ACA – Brisbane South*. Please note, any clients in Logan River Valley can only be seen after changing the outlet to *Dept of Health QLD – ACA – Logan River Valley*.

1.6.3 Client Record

The client record looks like this:

The screenshot displays the 'Client Record' interface. At the top, a teal header bar contains client information: 'Mr Firstname Lastname (Preferred Name)', 'Male, 100 years old, 1 January 1925, AC12345678', 'Unit 1, Lot Number 1 Streetname Street, Eight Mile Plains, QLD, 4113', and 'Primary contact: Contact Name (Supporter, Child) - 0400 111 111'. Below this is a 'View support network' link. The main content area has a 'Client summary' tab selected, showing a 'Comprehensive Assessment' finalised on 1 January 2026. Below this, there are sections for 'Recommendations and approvals' (listing 'Help at home - Entry level support' and 'Allied health and therapy - Occupational therapy') and 'Service delivery status' (listing 'Ended on 1 January 2025' and 'Referral issued on 1 January 2026').

The clients name, nickname(s), age, date of birth, AC number, address and primary contact can be seen in the top bar for easy viewing.

Please note the following tabs and their purposes:

- Client summary
 - Provides a summary of the client's current and previous assessments, status of current referral (e.g. triage accepted xx/xx/xxxx) and a list of approvals and associated providers.
- Client details
 - Provides a more detailed summary of the client's details e.g. Medicare card, other contact numbers etc.
- Support network
 - List of approved supporters and their contact information.
 - Please try other contacts if client is not answering or their phone seems disconnected.
- Approvals
 - Overview of the clients care approvals.
- Plans
 - Where the client's support plan and referral code letter can be generated.
- Attachments
 - Where any relevant documents (e.g. Approval Letter or HVRA etc) can be found and downloaded/uploaded.
- Notes
 - Complete client record, every interaction with client should be documented and noted in the record. Can be viewed to gain information about client situation and can aid in forward planning.

1.6.4 Review Requests

Support Plan reviews come from My Aged Care once requested by a client. This section shows our current and pending Support Plan Reviews. It looks like below:

Upcoming reviews

Currently viewing Dept of Health QLD - ACA - Brisbane South

Upcoming reviews Assigned reviews Review history

CARD LIST

Filter by

☐ Select all

1 to 6 out of 6 matching results

Last name	First name	Aged care user ID	Locality	Scheduled / Requested date	Requested Action Date	Assessed by	Urgency	Select

- Clients go from *Upcoming reviews* to *Assigned reviews* once they have been assigned to an assessor.
- You can navigate the various lists by clicking the relevant tab at the top. In the upper right, the current displayed outlet can be changed. The default outlet should be *Dept of Health QLD – ACA – Brisbane South*. Please note, any clients in Logan River Valley can only be seen after changing the outlet to *Dept of Health QLD – ACA – Logan River Valley*.

2 Appointments and bookings

2.1 Overview of referrals

Referrals are applications our clients put in to be assessed. They first go to the Intake section (*MAC -> Assessment referrals -> Incoming referrals*), the Intake team will triage them and assign them to the correct section (*MAC -> Assessment referrals -> Accepted referrals*).

Once in *Accepted referrals*, you can use the filters to narrow down specific referrals. E.g. to view the referrals for the East Hub, click on the expand button , then *MODIFY FILTER OPTIONS*, and click the dropdown box. Click *Assigned to* and *ADD FILTER*. A new box will appear with the filter. Click on *Assessor*, type in East Hub into the box that appears and select *East Hub*. Click *FILTER*. The list should now only show referrals assigned to the East Hub. This should be used when booking appointments in specific areas and can also be used to find specific clients.

Clients off the accepted list are called to book appointments with our assessors. Admin are to map clients in specific areas to specific assessors.

2.2 Planning appointments

We aim to only book two weeks in advance, and each pair of assessments on a given day for a given assessor should only have about 15 mins drive in between at maximum. When phoning a client to book an appointment it is ideal to have a few appointment options.

Print off a list directly from the *Accepted referrals* view on MAC with all the clients assigned to your given area (filtered), and you can make notes and cross clients off as you go. To print of this list, filter the *Accepted referrals* as shown in the example in 2.1 -> control P -> adjust print details and page zoom to fit personal preference -> enter or click print.

Please note that different types of appointments suit different clients. E.g. a client with mobility impairments would not be a good candidate for a clinic appointment etc.

Once you have selected a client to book, proceed to 2.3.

2.3 Calling clients

The process for calling clients is as follows:

- Find the relevant phone number on MAC (see 1.6).
- Call the client or carer listed as the primary contact. Ring twice as some of our clientele may have difficulty getting to the phone quickly.
- If no answer from client, try other contacts in their support network, especially if their phone does not ring or goes straight to voicemail.
- Leave a message consisting of the following:
 - Good morning/afternoon, this is <your name> from Metro South Aged Care Assessments. I am calling regarding booking in your aged care assessment appointment. Please return my call on <phone number> when you are available. Thank you!
- If the patient does not answer, try them again on 3 separate occasions including on different days. On the third try, we say if there is no contact by close of business then we will close the referral. We would then generate an unable to contact letter which will be given to the AC to send out at close of business.

When the client answers, follow this mini script:

- Good morning/afternoon, this is <your name> from Metro South Aged Care Assessments. Am I speaking with <client/carers name>?
- I am just calling regarding booking in your aged care assessment appointment.

Then you can provide options from the dates and times you planned earlier. Discuss the type of appointment and any possibilities (e.g. clinic and phone (see 2.6)).

- Once a date and time has been confirmed (for a home visit appointment), you can ask them the HVRA questions (see 2.4) and can provide the assessor's name.
- Please confirm with the client/carers that they will have their Medicare card and a list of any medication the client is taking ready, and that the assessor will discuss any health conditions or medical history, so having a record of those would be ideal.
- Please provide our generic phone number [REDACTED] in case they need to reschedule.
- Finish off by confirming the details and wishing them a lovely day!

Please remember to always maintain a respectful and informative phone manner and have patience with our older clientele. If at any point you are unsure of the answer to a question, just let the client know you will check with a supervisor and put them on hold. If at any point you are made to feel uncomfortable, hang up and report to a supervisor.

2.4 Home Visit Risk Assessment (HVRA)

The HVRA is a questionnaire that determines the risk level of visiting each client's home. Please see it attached below. The questions need to be asked when booking each appointment, and the client's details can be copied and pasted from MAC into the top right box.

It then needs to be signed and dated and then uploaded to the client's record in *Attachments* with the type 'Sensitive Attachments'. If any risk has been identified, the second page must be completed with an Area Coordinator.

A blank copy can be found at *DATA7 -> CommType -> ACAT -> Remote Assessment -> Forms -> MS ACAT Home Visiting Safety Risk Assessment*

 Queensland Government METRO SOUTH HEALTH AGED CARE ASSESSMENT TEAM Home Visit Risk Assessment		First name: Last name: DOB: Address: AC		
This form must be completed prior to home visit				
	YES	NO	N/A	COMMENTS
Is there suitable parking available?	☐	☐	☐	
Is the house number visible?	☐	☐	☐	
Are we able to access your home easily? (e.g., key safe access required, gated complex etc.).	☐	☐	☐	
Potential Risks				
Are there animals that may pose a risk (eg. dogs) on the property?	☐	☐	☐	
If yes, are they able to be restrained during home visits?	☐	☐	☐	
Do you have any issues with mobile phone or internet reception on your property?	☐	☐	☐	
Are you able to provide a smoke free environment when we visit you at your home?	☐	☐	☐	
Is there a risk of staff sustaining physical injury due to aggression or violence? (e.g., anyone <u>abuse</u> at the residence that presents a risk, weapons on the property etc.)	☐	☐	☐	
Are there any firearms stored on the property?	☐	☐	☐	
If yes, are they stored in a secure location with ammunition stored separately?	☐	☐	☐	
Is there a risk of staff sustaining physical injury due to unsafe steps, pathways, floor surfaces or other hazards?	☐	☐	☐	
Are there any other factors that may impact on staff safety?	☐	☐	☐	
Advise that the assessor will call to check for any illness prior to appointment from a no-ID number				
Additional Information:				
HAS A RISK BEEN IDENTIFIED? ☐ YES ☐ NO If YES, print and give to Area Coordinator or Director to complete next page If NO, print to PDF and upload as sensitive attachment to client's MAC record				
Name:		Date:		

Page 1 of 2

Home Visit Risk Assessment

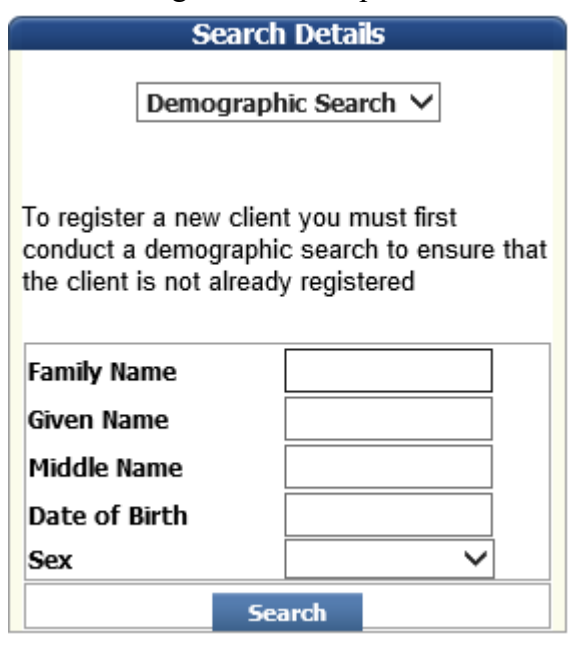
 Queensland Government METRO SOUTH HEALTH AGED CARE ASSESSMENT TEAM Home Visit Risk Assessment		First name: Last name: DOB: Address: AC				
Consequence of Risk Consideration of patient/client and staff safety is an essential part of patient/client selection. Due to the unpredictable nature of the home environment, it is essential that comprehensive risk assessment is completed prior. The assessment needs to identify any potential treatment and environmental risks. If a risk has been identified, the Queensland Health Risk Matrix (below) should be used to score the individual level of risk and implement an appropriate management plan. This must always be done in collaboration with your Line Manager. Refer to the MSH Risk Assessment Tool for criteria for WHS and Delivery of Safe Services.						
		Consequence				
		Negligible	Minor	Moderate	Major	Extreme
Likelihood	Almost certain	☐ Medium (7)	☐ Medium (7)	☐ High (17)	☐ Very High (23)	☐ Very High (25)
	Likely	☐ Medium (6)	☐ Medium (10)	☐ High (16)	☐ High (20)	☐ Very High (24)
	Possible	☐ Low (3)	☐ Medium (9)	☐ High (15)	☐ High (18)	☐ High (22)
	Unlikely	☐ Low (2)	☐ Medium (8)	☐ Medium (12)	☐ Medium (14)	☐ High (21)
	Rare	☐ Low (1)	☐ Low (4)	☐ Low (5)	☐ Medium (13)	☐ High (19)
Rating		Low (1-5)	Medium (6-14)	High (15-22)	Very High (23-25)	
Management Action (circle relevant)		PROCEED WITH HOME VISIT		DETERMINE IF HOME VISIT SAFE		DO NOT HOME VISIT
		DO NOT HOME VISIT		DO NOT HOME VISIT		
Complete Safety Management Action Plan						
Safety Management Action Plan (must be completed for all identified risk)						
☐ 2 staff to home visit ☐ Offer client phone contact only ☐ Comments / Other:						
Name:			Designation:			
Signature:			Date:			
If there are any changes to a patient/client's home environment (i.e. change of address, change in behaviour), a new MSH Home Visit Risk Assessment Tool must be completed.						

Page 2 of 2

2.5 CHIMS process

Once a date and time has been confirmed from the phone call, the appointment needs to be booked into CHIMs. To do so open CHIMS (as shown in 1.3), then follow the steps below:

- Click on *File*
- Then in the dropdown, click on *New Search*
- The following screen will open:



- Search with clients DOB but can use the client's names if required.
- If the client's record already exists continue to 2.5.2, double check that all the details are up to date. If no record exists, click on *Add New Client* and see 2.5.1.

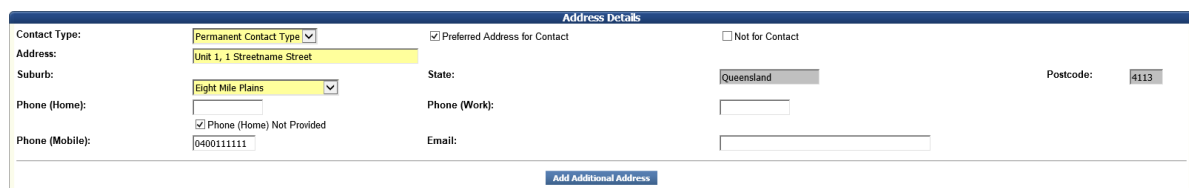
2.5.1 Creating new CHIMS client

The sections in yellow are required. Most of these details can be found in the client record under *Client Details*.

- Enter the client's details as shown below:



- Fill out the address and contact details as below.



- Fill out the demographic details as shown below as relevant to the client. If the Medicare expiry is known, please fill it in. If unknown, it can be left blank:

Demographic Details

Country of Birth: Australia Religion:

Indigenous Status: Neither Aboriginal nor Torres Strait Islander origin ASSI: Non South Sea Islander Origin

Preferred Language: English Only Interpreter Required: ☐

Marital Status: Never Married

Medicare Eligibility: ☒ Eligible for Medicare ☐ Not Eligible for Medicare ☐ Not Stated/Unknown ☐ Reciprocal

Medicare Number: 1111/1111/11 Medicare ID: 1 Medicare Expiry Date:

OR ☐ Card Unsigned

Client has consented to Bulk Billing? ☐

Pension Status: Pension Number: Pension Expiry Date:

Private Health: ☐

Does the client consent to being contacted for feedback? ☐

Additional Billing Information: 500

Additional Registration Information: 500

Client information is also stored in the following systems:

☐ ATODS-IS ☐ ACE ☐ Practix ☐ TRACCS ☐ CARE-PRO ☐ CIMHA

Next>> Complete>> Cancel>>

- Click *Next>>* and then click on *Contacts*. Add in any relevant Supporters or family as required.
- Continue to 2.5.2

2.5.2 Creating Appointment



- To book in the appointment, click on the green man
- Fill in the details as below. Select the right date, time and select the correct assessor from the *Select Clinician* dropdown box. Note that the time is in 24hr format:

Appointment Details

Client Appointment

Stream: Aged Care Services

Service: ACAT

Team: ACAT - Brisbane South

Client: 690977 Search

Firstname Lastname

Interpreter Booked: ☐ Yes

Focus: Direct Client Activity

Mode: In Person

Location: Domestic/Home Setting

Domestic/Home Setting

Nurse Navigation Phases: Select Phase

Distance: Km

Travel Time: Minutes

Type: ☒ New Appt ☐ Review Appt ☐ Recall Appt

Appointment Time (Please Select or Enter Times)

Start: End:

Appointment Dates: << Monday, December 29, 2025 >>

December 2025							January 2026						
Su	Mo	Tu	We	Th	Fr	Sa	Su	Mo	Tu	We	Th	Fr	Sa
30	1	2	3	4	5	6	28	29	30	31	1	2	3
7	8	9	10	11	12	13	4	5	6	7	8	9	10
14	15	16	17	18	19	20	11	12	13	14	15	16	17
21	22	23	24	25	26	27	18	19	20	21	22	23	24
28	29	30	31	1	2	3	25	26	27	28	29	30	31
4	5	6	7	8	9	10	1	2	3	4	5	6	7

Diary: ☒ Clinician ☐ Clinic

Clinicians in Attendance: Select Clinicians...

Comments:

- If an interpreter is required, click the tick box next to *Interpreter Booked* (green arrow below) and follow the steps in 2.7 once the CHIMS appointment has been created.

Appointment Details

Client Appointment

Stream: **Aged Care Services**

Service: **ACAT**

Team: **ACAT - Brisbane South**

Client: **690977**

Firstname Lastname

Interpreter Booked: ☐ Yes ☒ No

Focus: **Direct Client Activity**

Mode: **In Person**

Location: **Domestic/Home Setting**

Nurse Navigation Phases: **Select Phase**

Distance: Km

Travel Time: Minutes

Type: ☒ New Appt ☐ Review Appt ☐ Recall Appt

Appointment Dates:

December 2025							January 2026						
Su	Mo	Tu	We	Th	Fr	Sa	Su	Mo	Tu	We	Th	Fr	Sa
1	2	3	4	5	6	7	8	9	10	11	12	13	14
15	16	17	18	19	20	21	18	19	20	21	22	23	24
28	29	30	31	1	2	3	25	26	27	28	29	30	31

Diary: ☒ Clinician ☐ Clinic

Clinicians in Attendance: **Select Clinicians...**

Comments:

Appointment Time
(Please Select or Enter Times)
Start: End:

- Once filled in, click *Submit* and the appointment should appear in the multiple view.
- Double check that the details are all correct.

2.6 MAC process

To finalise the appointment in MAC, follow these steps:

- Navigate to Assessment referrals and find the client again. Open *Attachments* and attach the HVRA as a 'Sensitive Attachment' (see 2.4).
- Open notes and copy the relevant appointment booking template. Fill this in with the details required and paste in as new note under Client Story.
- If an interpreter was booked, state this in the MAC booking note along with the language of the interpreter.
- If a family member/supporter will be present at the assessment, make a note of this as well within the MAC booking note.
- Navigate back to the client in Assessment referrals and click on *Reassign*. Assign the client to the relevant assessor (using control-F may be ideal).

2.6.1 Phone assessments

- For phone assessments, no HVRA questions are required to be asked or completed. Please ask the client/carer what phone number they prefer to be called on for the assessment and make a note of this when making the MAC note.
- When booking CHIMs, please select location as *Other Setting* and then in the second dropdown select *Telephone Consultation*.
- When making the MAC note, please copy the relevant phone assessment template and add additional details as necessary.

2.6.2 Clinic assessments

- For clinic assessments, no HVRA questions are required to be asked or completed.
- When a client has accepted a clinic assessment appointment there is a template letter that is filled in by Admin Officers with the client's details, this letter is then posted/emailed to the client. This clinic letter has information on it for the client, location and parking etc.
- When booking CHIMs, select location as *QEII Community Health Centres* and then in the second dropdown select the relevant clinic.
- When making the MAC note, copy the relevant clinic assessment template and add additional details as necessary.

2.7 Booking interpreters

- Open Primaxis from the desktop and log in:

- Click on *Request Interpreter*. Search by the client's UR number from CHIMS, if none found click on *Create Booking*. Input the correct details as shown below:

- Ensure that the assessors name, language required, and notes to interpreter have been inputted correctly. Now fill in the patient's details in the boxes shown below.

- In the requested by section, type your name, the generic [redacted] email and the generic phone number
- Click Save.
- Print out the summary and keep it in your interpreter management clipboard for easy access.

2.8 Cancellations and rescheduling appointments

2.8.1 Cancelling

To cancel an appointment, open CHIMS (see 1.3), find the appointment in the multiple diary view. Double click the appointment, which opens the appointment summary. In the dropdown for Appointment Status (see below), change to cancelled by client or clinician as relevant. Click *Submit and Close* and exit CHIMS. Go to the clients record on MAC, fill out the cancellation note template and paste this note into record. Navigate to Assessment referrals and filter by the assessor. Reassign the referral back to the relevant area (unassign from the assessor). The appointment will need to be rescheduled at some point so make a note somewhere.

The screenshot displays the CHIMS Appointment Summary form. At the top, it shows 'Date Created: 03/12/2025 03:56:48 PM' and 'Appointment Created By: [redacted]'. Below this, 'Date Modified: 23/12/2025 10:21:02 AM' and 'Last Modified By: [redacted]'. The form fields include 'Time: 09:30 - 11:00', 'Appointment Status:' (with a dropdown menu open), 'Client: [redacted]', 'Clinicians/Clinics: [redacted]', 'Location: Domestic/Home Setting', 'Mode: In Person', 'Focus: Direct Client Activity', 'New or Review: NEW', and 'Comments:'. The dropdown menu for 'Appointment Status:' is open, showing options: 'Select One', 'Appointment Confirmed', 'Arrived', 'Booked' (highlighted), 'Cancelled Data Error', 'Cancelled by Client', 'Cancelled by Clinician', 'Completed', and 'Did Not Attend'. At the bottom, there are buttons for 'Print Appointment Letter', 'Print Travel Sheet', 'Edit', 'Treatment Description', and 'Delete'. The footer shows 'Date Created: 24/12/2025 08:43:54 AM' and 'Appointment Created By: [redacted]'.

2.8.2 Rescheduling

To reschedule an appointment, find the client an appropriate rescheduled appointment date and time and then cancel as above without the MAC note. Use the relevant rescheduling MAC note instead with the new appointment details. Rebook in CHIMS with the new details as if booking a new assessment. Navigate to Assessment referrals and reassign the client to the new assessor. No new HVRA required if just rescheduling unless address has been changed.

3 Letters and mail

Check the letters folder every morning to see what mail needs to go out. The green mail crate needs to be brought down to reception every day by 1pm.

3.1 Action sheets

Each finalised referral or SPR will lead to the assessor filling out an action sheet and placing it in a relevant folder separated by the area. The action sheet contains information about what paperwork to send out, who to send it to and what method to send it by. See section 3.7 to view a blank copy of the action sheet.

Action sheets can be found as follows:

DATA7 -> CommType -> ACAT -> Remote Assessment -> Admin Action Sheets – Community AXs or Admin Action Sheets – Hospital AXs.

On the left under ‘Client Correspondence’ are the client’s personalised letters, which are generated on MAC with the client details added on with their specific approvals and situation. They are as follows:

- The approval letter is a letter containing the notice of decision made by the assessor.
- The support plan is another letter with all the approved services and support strategies for that client.
- The referral code letter is a summary of the client’s approvals and their relevant approval codes all on one page.
- The non-approval and SPR letter are just variations of the approval letter.
- The triage letters are handled by the triage team and can be ignored at this stage.
- Unable to contact letters are sent out after three unsuccessful attempts to contact a client.

On the right under ‘Information to be Sent’ are the brochures published by MAC that provide information on approvals and services provided.

Where to find each can be found below.

3.1.1 Approval letter, non-approval letter and SPR letter

All found under the attachments section of the client record as the assessor will upload it there.

3.1.2 Support plan

Usually, the support plan is also found under attachments. In some instances, however, the support plan may not have been uploaded. It can be manually generated by navigating to *Plans -> Support Plan -> Print Copy of Support Plan*. This generates a PDF that can then be saved and uploaded to attachments or printed and sent out.

3.1.3 Referral code letter

The referral code letter needs to always be manually generated from the client record. This can be achieved by navigating to *Plans -> Support Plan -> Manage services and referrals -> scroll down to the bottom -> GENERATE REFERRAL CODE LETTER*. This usually redirects

you to the reports page, and the referral code letter will be processing. After a few seconds, clicking on *Status* should refresh the list, allowing you to click [view](#) which will download a copy to be saved and printed.

3.1.4 MAC - DPS Perm and Home Care

Hard copy only and found in the shelves in front of the Area Coordinator offices.

3.1.5 MAC – Carer Gateway Fact Sheet

DATA7 -> CommType -> ACAT -> Resources -> ACAT Information Leaflets -> Carer Gateway Factsheet

3.1.6 MAC – Brochures

The rest can all be found by searching My Aged Care into Google -> scroll to the bottom -> *Resources* -> Scroll down to Booklets and Brochures to learn more.

The relevant booklet of each action sheet entry has been shown below:

Action Sheet	MAC Brochure
MAC steps-to-enter-aged-care-home	Booklet: Steps to enter an aged care home
MAC your guide to CHSP	Booklet: Your guide to the Commonwealth Home Support Program
MAC your guide to SaH services	Booklet: Your guide to ongoing Support at Home services
MAC your pathway to accessing SaH	Brochure: Support at Home program
MAC your guide to Restorative Care Pathway	Booklet: Your guide to the Restorative Care pathway

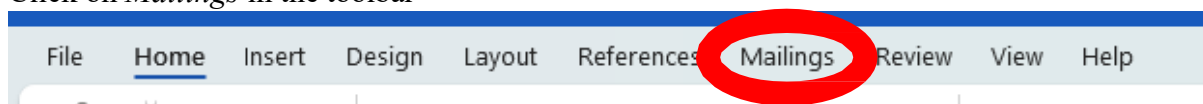
Important: These can be saved onto your desktop for faster access as letters are an everyday task.

3.2 Mail

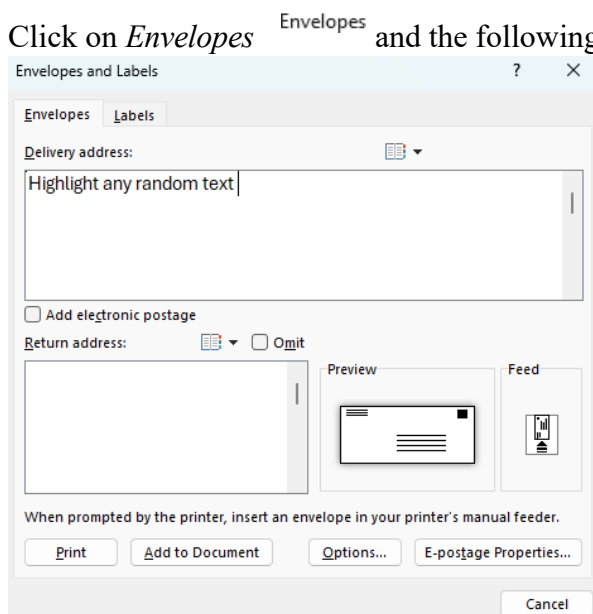
For mailing, the correct type of envelope needs to be used depending on contents (See 3.2.1). Consult each section for each type of required communication:

For most mail without any of the large MAC booklets, the medium sized A5 envelope is standard to send out. These can be printed with the client's address using Word's Mailings feature. To do so follow these steps:

- Stamp the inner side (side with the flap) of the envelope with our return address stamp (can be done in bulk)
- Place these into the printer in the designated envelope tray with the correct orientation
- Have the client's name and address ready
- Open any word document, using the AO notes template sheet is standard practice
- Highlight any random text
- Click on *Mailings* in the toolbar



- Click on *Envelopes* and the following window will open.



- Replace the random text with the name and address of the client in the following format:
Mr/Mrs/Ms/Dr Firstname Lastname,
ACA Retirement Village,
Unit 1, 1 Streetname Street,
Eight Mile Plains, QLD, 4113
- Click print or enter and the envelope should print with the address on the correct side.

For any outgoing mail that does contain the larger MAC brochures or DPS guides, the A5 envelopes CANNOT be used. The larger A4 envelopes must be used, and the name and address must be written by hand in the same format on the correct side. The return address must also be stamped.

The relevant paperwork can then be placed into these envelopes, sealed and then placed into the green mail tray.

3.2.1 Surveys for mail

Ensure all outgoing paperwork has a client satisfaction survey and return envelope enclosed. Return envelopes require printed labels that can be bulk printed and stuck onto small envelopes. Surveys can also be bulk printed. They can be found in:

DATA7 -> CommType -> ACAT Admin -> SURVEY INFO

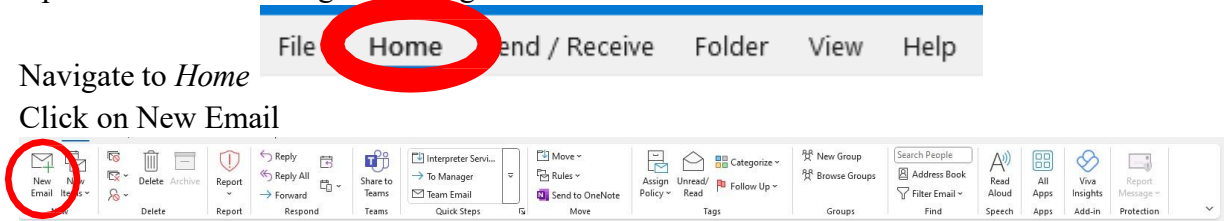
Look for 'Client Satisfaction Survey Clinical'.

3.3 Emails

Emails can be sent with attachments from the generic ACA email. To do so, follow these steps:

- Download and name all relevant paperwork. Use a consistent naming framework e.g. ACA Support Plan – Lastname Firstname + ACA Approval Letter – Lastname Firstname
- Open Outlook and navigate to the generic email

- Navigate to *Home*
- Click on New Email



- Ensure the email is from [REDACTED]
- Enter the recipients email in the 'To' section
- Enter the subject as 'ACA Documents' or similar
- Attach all the relevant documents
- Copy and paste the email template from the AO template sheet and fill in the details
- Add your signature
- Click *Send*
- Monitor emails for a few minutes in case it bounces back

3.4 Fax

Some GPs still use fax for communication. To send support plans via fax follow these steps:

- Download the support plan and save it
- Open the Fax template (*DATA7 -> CommType -> ACAT -> TEMPLATES -> QH Fax Template*)
- Fill out the details of the recipient (Fax number, Dr name, date etc)
- Write the subject as 'ACA Support Plan for Firstname Lastname'
- Check the number of pages of the support plan and add 2. This is the number of pages (inclusive). Add this next to Pages.
- Print off this fax template
- Place the fax template in front of the support plan
- Go to the printer and select the fax option
- Tap the keypad option and type in the recipient's fax number with a 0 in front
- Place the fax template and support plan into the copying/fax tray
- Tap settings and choose the relevant options e.g. double sided, orientation etc
- Press send and a blue light should start flashing on the fax communicating light on the side of the touchpad
- The printer will print off a page with the status of the fax. If it went through, the status would say 'OK'
- If there was an error, try another printer/fax machine

3.5 Steps to send out letters

Check the action sheet for what documents are required and in what format. Mail needs to be printed, and emails just need to be downloaded. See 3.1 for where to find each form of letter and brochure.

3.5.1 Client correspondence

Send the required email/mail to the client with the address stated in the box (most likely says As Above so refer to the top right client summary). Client gets all the paperwork that is ticked.

3.5.2 GP correspondence

At the bottom left of the action sheet is the GP correspondence, in which some clients would like a copy of their support plan be sent to their GP. GPs can only receive support plans. If this section is ticked, the support plan can be sent out to the GP in the chosen format.

3.5.3 Copies with consent

Copies of all paperwork can also be sent to supporters as chosen by the clients and assessors. Same process as with client correspondence.

3.5.4 Hospital correspondence

Copies of paperwork can be sent to Hospitals. Typically, this is already sent to the hospital by assessors and will be indicated as such, but if not, follow the instructions that have been indicated by the assessor.

3.6 Hospital Letters

Hospital letters are almost the same as community ones but require more priority. Please check the hospital letter folder every morning and ensure they get sent out as these clientele may be in time dependent situations.

They follow the same process as community letters but once they have been sent out, please navigate to *DATA7 -> CommType -> ACAT -> Remote Assessment*.

There will be two folders there, PRIVATE HOSPITALS – REFERRAL INFORMATION and PUBLIC HOSPITALS – REFERRAL INFORMATION.

Please find the client in these folders depending on what hospital they were at and move their entire folder into the xx. MONTH COMPLETED folder of the current month.

3.7 Blank Action Sheet

A blank action sheet can be found at *DATA7 -> CommType -> ACAT -> Remote Assessment -> Forms -> Admin Action Sheet Nov 2025 BS ACA*

 Queensland Government BRISBANE SOUTH Aged Care Assessment Organisation Admin Action Sheet		
Client Correspondence ☐ Triage Letter ☐ Approval Letter ☐ Support Plan ☐ Referral Code Letter ☐ Non-Approval Letter ☐ SPR Letter ☐ Unable to contact letter ☐ Action Required: ☐ Sent ☐ To be sent ☐ To be faxed/emailed NAME: ADDRESS: SUBURB: TO BE FAXED/EMAILED TO:		Information To be Sent ☐ DPS Perm Care (post only) ☐ DPS Home Care (post only) ☐ MAC steps-to-enter-aged-care-home ☐ MAC your guide to CHSP ☐ MAC your guide to SaH services ☐ MAC your pathway to accessing SaH ☐ MAC your guide to Restorative Care Pathway ☐ MAC Carer Gateway Factsheet ☐ Additional Info:
Copy with consent: SUPPORTER Action Required ☐ Sent ☐ To be sent ☐ To be faxed /emailed Name: Address: Suburb: To be faxed/emailed to:		Copy with consent: Action Required ☐ Sent ☐ To be sent ☐ To be faxed /emailed Name: Address: Suburb: To be faxed/emailed to:
Copy with consent: Action Required ☐ Sent ☐ To be sent ☒ To be faxed /emailed Name: Address: Suburb: To be faxed/emailed to:		Hospital Correspondence Action Required ☐ Sent ☐ To be sent ☐ To be faxed /emailed Name/title: Facility: To be faxed/emailed to:
Name: Designation:		Signature: Date:

ACAT Admin Action Sheet

☐

4 Phones, emails and responding to enquiries

4.1 Phone and email process

In the morning huddle there will be a discussion about time slots for each AO to cover the phone. Adjust according to staffing, there is a template with the time slots based on number of staff next to the phone.

During your slot:

- Check for voicemails that were left within allocated time and follow up as necessary.
- Check for emails that were left within allocated time and follow up as necessary
- Keep record of these messages/emails in case of follow up in future

It is suggested that you take notes for each message. Vital information would be the client's AC number and phone number. Please note the phone number is displayed on the screen for each message (excluding private numbers). Make a note of as much information as you can to simplify the process of understanding each situation.

If you are first on the phone, please check the phone with extension 9490 as it contains the overnight messages when switchboard turns off the main phone. Press the tab on the phone next to the 'V' and type 0000 as the password. These messages also need to be followed up on if required.

Below are some common scenarios that arise when monitoring the phones.

4.2 Scenarios

4.2.1 Client enquires about wait time

Send the client a text message from the allocated mobile phone utilising the template for the message. The message must not indicate timeframes and must contain a do not reply to message at the bottom.

4.2.2 Client wants priority assessment

If the client already has a referral, call client back to gain some more insight as to why and let them know that you will pass this onto an Area Coordinator. Do not provide a time frame for this.

Copy and paste the 'Forwarded to AC' note from the template into the Notes section of the client record and add the client/carer's phone number.

Print off the first page of the Notes section of the client and write a quick summary message of the why you discussed earlier on the top right under the primary contact. Place this in the relevant tray based on area for the Area Coordinator to follow up later.

This is the same for Home Support Assessments, place these in the HSA team leader's tray which is found in the HSA pod. Reminder to also use the forwarded to HSA note.

4.2.3 Client/Supporter wants copy of paperwork or referral code

Call client back and let them know to call MyAgedCare on 1800 200 422 as they can provide the paperwork. Liaise with the Area Coordinators if this is an issue for the client as we can send it out if required.

4.2.4 Client is returning a call to someone

If the client is returning a call to intake:

- Copy and paste the 'Return call to Intake' note from the template into the client record and print this off. Place in the intake tray.

If the client is returning a call to an Assessor:

- Copy and paste the 'Return call to Assessor' note from the template into the client record. Send an email to the relevant assessor with the client's information and let them know they have called back.

If the client is returning a call to an Area Coordinator:

- Copy and paste the 'Return call to Area Coordinator' note from the template into the client record and print this off. Place in the relevant AC's tray.

If the client is returning a call to the Home Support Assessment team:

- Copy and paste the 'Return call to HSA' note from the template into the client record and print this off. Place in the HAS or you could email depending on the situation.

4.2.5 Client is wanting to add a supporter

Unfortunately, we do not have the authority to do this so the client must ring MyAgedCare on 1800 200 422.

4.2.6 Other

Liaise with other team members or with Area Coordinators for more complex situations. Do not be afraid to ask questions!

4.3 Email responses

All emails to the generic email 'marked green' or marked as 'COMMUNITY ADMIN TO ACTION' must be actioned by the ACA Administration team.

When responding to emails, utilise the generic email reply found in the AO template (unless a specific reply was required). The email can then be actioned, which can be forwarding, noting or replying to the email.

Once email has been actioned, double click the email, and a new window will open that will allow you to edit the email. You can now edit the subject line to highlight what was done/actioned. E.g. 'Forwarded to Assessor – *(Your Initials)*: Original subject'.

Include your initials in this addition so that it can be followed up later if required. Press Ctrl and S on your keyboard to save the new subject line. You can now drag this email into the relevant folder. If the email deals with clients and any carers, they go into 'Community'. If the email is internal based, place it into 'ACA Admin'. Interpreter emails are placed into 'Interpreter Services' and any email where no action was taken/required are placed into 'No Action Required'.

5 Miscellaneous tasks

These tasks can be completed during downtime.

5.1 Surveys

After clients have their assessment, they are sent a satisfaction survey. Once they mail it back, we need to log them in the spreadsheets. Returned surveys can be typically found above the phones in their envelopes. These need to be opened and organised into clinical and non-clinical.

The survey folder can be found in:

DATA7 -> CommType -> ACAT Admin -> SURVEY INFO

From here, select the correct financial year, e.g. 25-26, and then the correct month range. Start with the clinical surveys, hence, open the 'Clinical - Client Satisfaction Survey Tool' and the 'Comments Spreadsheet'.

For each returned survey, you will need to fill in the survey tool exactly how it is on the returned survey, ensuring no identifiable information is saved. Add any comments to the bottom of the survey tool and copy it over to the comment's spreadsheet. In the comment's spreadsheet, date and initial each comment that you entered. Press submit on the survey tool and the form goes back to default (except the comment, you will need to delete this manually). Repeat this for every returned survey.

Close the 'Clinical - Client Satisfaction Survey Tool' and open the 'Non-Clinical - Client Satisfaction Survey Tool'. Repeat the above process for all the non-clinical surveys.

Ensure the work is saved by pressing Ctrl and S on all three documents.

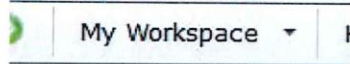
5.2 RiskMan

Please see the attached guide on the next page.

ENTERING COMPLIMENTS INTO RISKMAN

- Scan hard copy of the compliment and save to:
Metro South > Metro South Health @ Home > Compliments

- Log into Riskman using the search bar
- Click on My Workspace > New > Feedback



- A new form will Open "*Feedback Entry*"

Feedback Entry

Submit this form to record the feedback.
You will be able to modify this page once it is submitted.

- Complete all the mandatory **yellow** fields
 - Service Region – Metro South Health @ Home
 - Facility – Eight Mile Plains Community Health Centre
 - Level – Level 1
 - Ward/Unit – Aged Care Assessment Team (ACAT) Eight Mile Plains
 - Division – Metro South Health@Home
 - Service/Stream – Aged Care Assessment Team (ACAT)
- **Select "*Save my location details for future use*"
- Complete sub form "*when was feedback received?*" This is the date the compliment was received. USE CALENDER DROPDOWN
Is this related to a specific event? NO

A screenshot of a form section titled 'When was Feedback Received?'. It contains two date fields: 'Date entered in RiskMan' and 'Date received', both set to '29 Apr 2022'. Below these is a question 'Is this related to a specific event?' with radio buttons for 'Yes' and 'No', where 'No' is selected.

- Complete sub form "*Person providing feedback*"
Is this employee feedback? NO
Feedback Source – USE DROPDOWN
Does this relate to a patient/client – YES

A screenshot of a form section titled 'Person Providing Feedback'. It contains three questions: 'Is this employee feedback?' with radio buttons for 'Yes' and 'No' (where 'No' is selected), 'Feedback source' with a dropdown menu, and 'Does this relate to a patient / client?' with radio buttons for 'Yes' and 'No' (where 'Yes' is selected).

- As this relates to a patient/client an additional sub form will open “*Patient/Client details*”
Complete all fields as required. If the URN is available free test into *Person Identifier #* and click in the “*Retrieve patient details*” button. Another window will pop up and provide the patients demographics. Check the patient details are correct and click “accept” and the fields will pre-populate with the data. IF IT IS NOT CORRECT, exit out of the window and select “*Person identifier # not known*”

Person Providing Feedback

Is this employee feedback? ☐ Yes ☒ No

Feedback source ^{*}

Does this relate to a patient / client? ☒ Yes ☐ No

Patient type ^{*}

☒ Person identifier # not known / applicable

Title

First name ^{*}

Surname ^{*}

Alias

Date of birth ^{*}

Gender ^{*} ☒ Male ☐ Female ☐ Indeterminate ☐ Not stated

Add contact details? ☐ Yes ☒ No

Aboriginal or Torres Strait Islander origin? ^{*}

Preferred contact name? ☒ Yes ☐ No

Age ^{*}

- Sub form “*Communication type*” is a DROPDOWN with multiple options available. Select the field that is suitable to the data you are entering.

Communication Type

Communication type

Please specify

Original feedback document received

- Sub form “*Type of feedback*” Classification is TREATMENT

Select items

Complaint Compliment General feedback

Access and timing Clinical safety Communication Complaints management

Diagnosis Discharge and transfer Humaneness/caring Management of facilities

Medication Patient rights Policy Professional conduct

Records Treatment

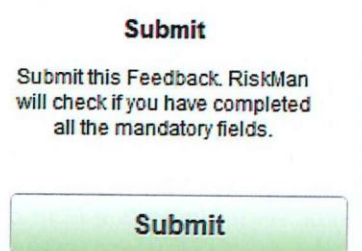
✓ ✗ ↺

- Sub form “*Compliments*”
 Select “Add compliment”
 ID – Prefilled
 Category * Access and Timing – Prefilled
 Other Category – Other – USE DROPDOWN
 Named Person – Leave blank
 Line Manager – Leave blank
 Comment – Compliment

- Sub form “*Details*”
 Summary – Metro South Health @ Home Compliments
 Detail – Write a brief summary of the compliment received.

- Sub form “*Documents*”
 Add the document you scanned in earlier, making sure it's the correct patient selected.

- Once all the fields have been completed, select “*Submit*”

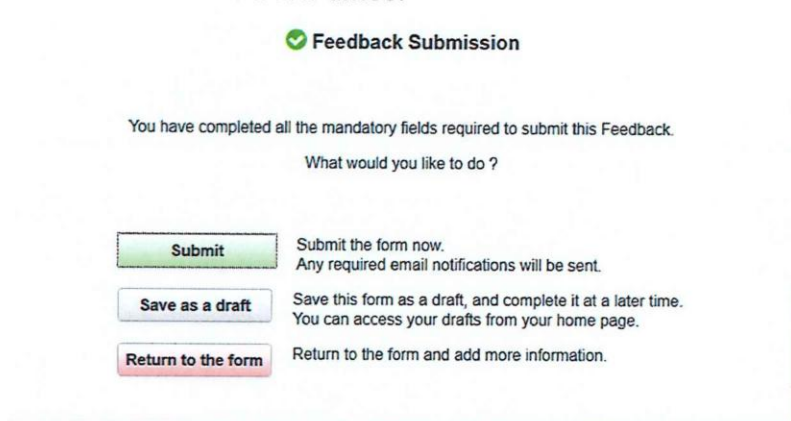


Submit

Submit this Feedback. RiskMan will check if you have completed all the mandatory fields.

Submit

- A window will pop-up asking for confirmation for the record to be submitted. Choose “*Submit*” If there is anything missed it will take you back to the section that needs to be filled and flash a few times.



Feedback Submission

You have completed all the mandatory fields required to submit this Feedback.

What would you like to do ?

Submit	Submit the form now. Any required email notifications will be sent.
Save as a draft	Save this form as a draft, and complete it at a later time. You can access your drafts from your home page.
Return to the form	Return to the form and add more information.

If further clarification is required, please contact your local CLO officer.

- Print the form to attach to the original hard copy of the compliment, attach all compliments for the Month, to be retained in NUM's folder.
- Records details on the “*Riskman Data Entry*” spreadsheet
TransitionCare > Rehab@Home Admin > Compliments

**** Coming in 2023 – “Have your say cards” will have a new section added asking if clients want to be involved in community activities. Riskman will have a section added under ‘Contact details’ to either say yes or no to being involved. ****

5.3 Ordering MAC brochures

As brochures are required regularly for clients it is important to ensure the stock levels are maintained and if required ordering more when necessary.

To order, navigate to the My Aged Care Website -> scroll all the way down -> *Resources* -> Hard copy resources -> *online order form*

- Select the organisation type as Assessment organisations and press Start.
- Select the quantity of each booklet required and add them to the cart.
- Once finalised, scroll down to the bottom and click View Cart.
- Click - continue to shipping details.
- Fill in the details with the Eight Mile Plains Community Health Centre Address as below and your name. Organisation name is Brisbane South ACA. You may put your desk extension for your contact phone.

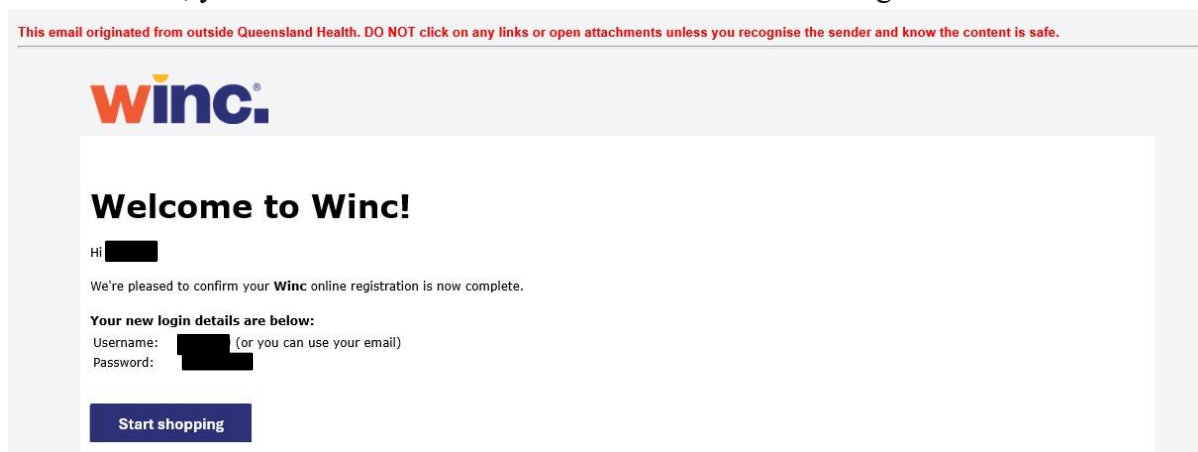
Eight Mile Plains Community Health Centre,
Brisbane Technology Park,
51 McKechnie Dr,
Eight Mile Plains, QLD, 4113

- Do the Captcha and click Place Order.
- Collect mail from downstairs once it arrives.
- Unbox and organise based on type.
- Place into correct area.

5.4 Stocktake and WINC ordering

There is a spreadsheet with all the Winc purchasable items available in the office supply cupboard. Keep track of the quantity of these items available and reorder them once only a few are remaining to keep constant stock available for use. The ordering process has been outlined below:

To begin, request access and an account through your line manager. Once this process has been finalised, you will receive an email that looks like the following:

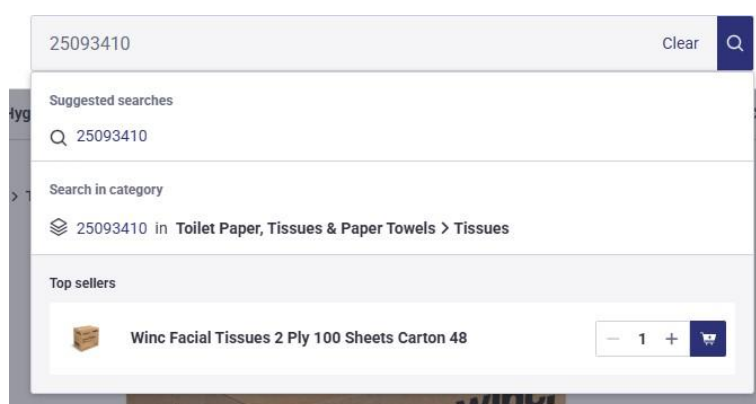


Important: Please save this email in a folder for easy access your login details and/or the start shopping button

Click the *Start shopping* button to begin your order. Click Log in (top right corner) and enter your personal login details.

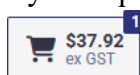
Otherwise, you can navigate to <https://www.winc.com.au/> and click Log in (top right corner) and enter your personal login details.

You should now be logged in and ready to order. You can browse products using the search function. Previously purchased items will have a 'Reorder code' on them, which can be entered into the search bar to find easily e.g. Tissue boxes can be reordered with the code 25093410:



Click on the correct item, adjust the quantity as required and then click *Add to Cart*. Once all

items have been added, click on the cart:



Click on *Review cart*, check that the items and quantities are correct. Click on *Continue to Checkout*. Enter your name, email, and [REDACTED] as the telephone. Enter the approver as the correct financial approver. As of January 2026, for the Aged Care Assessment service, the approver is [REDACTED] Click *Continue to Shipping Address*.

Select the following as the address:



ACAS QE CW

8 MILES PLAINS CMTY HLTH CTR, 51 MCKECHNIE DR, EIGHT MILE PLAINS, QLD 4113

[Edit](#)

Select *Order Now*. Then click *Continue to payment* and select *Charge to Current Account*.

Finalise order by clicking on *Submit order for approval*.

Typically, you will receive an email from reception indicating that the delivery has arrived. Please collect and unpack the delivery at your convenience. You will have to sort the delivery and it's items into the stationary cupboards as required.

6 ACAT Process Template and Notes

A copy of the template with the admin templates can be found in *DATA7 -> CommType -> ACAT Admin -> ACAT A03 Notes – Template*. A copy has been attached below.

AO3 Notes/Template for MAC:

*Replace anything in italic brackets e.g. (*name*)

**Replace XX with initials

***Underlined means optional note

Incoming Phone Messages:

Area Coordinator	<u>(Return)</u> Call from (<i>name</i>) Phone number (<i>number</i>) Action: Forwarded onto Area Coordinator. XX Admin
Intake	<u>(Return)</u> Call from (<i>name</i>) Phone number (<i>number</i>) Action: Forwarded onto Intake. XX Admin
Assessor	<u>(Return)</u> Call from (<i>name</i>) Phone number (<i>number</i>) Action: Forwarded onto Assessor. XX Admin
HSA	<u>(Return)</u> Call from (<i>name</i>) Phone number (<i>number</i>) Action: Forwarded onto HSA. XX Admin

Unable to Contact:

General	Phone call to (<i>client/carer</i>). No answer, message left to call back. (<i>Text sent</i>). XX admin
For appointment: First attempt Second attempt Third attempt	Phone call to (<i>client/carer</i>) to make an ACA appointment x (<i>number of attempts</i>). No answer, message left to call back. (<i>Text sent</i>). XX admin
After three attempts	Three attempts to contact client to make an ACA appointment. Message sent (<i>dd/mm/yyyy</i>) advising that if no response received by close of business (<i>dd/mm/yyyy</i>), then the referral would be rejected. Advised client that if referral reactivation is required then please contact MAC. XX admin

	- Pension/ Health care or DVA card - List of current medications - GP health summary and other current medical reports Cancellation: For any cancellation or rescheduling enquiries, please phone (ACA) Aged Care Assessment Team on 3156 9400. Kind regards,
--	--

Cancellations/Rescheduling:

Cancellation	Appointment on (<i>dd/mm/yyyy</i>) (<i>time</i>) cancelled by (<i>client/carer</i>). Await contact to reschedule. XX admin
Cancelled and Rescheduled	Appointment on (<i>dd/mm/yyyy</i>) (<i>time</i>) cancelled by (<i>clinician</i>) or (<i>client/carer</i>). New appointment scheduled for (<i>dd/mm/yyyy</i>) at (<i>time</i>) with (<i>clinician</i>). XX admin

Letters/Mail Out:

Action Sheet Completed	ACA Admin Action Sheet completed and sent as per request by Clinician. XX Admin
Email Out	Good morning, (<i>name</i>). Please find attached the Aged Care paperwork for (<i>client</i>) for your records. Kind regards,

Email Reply:

Generic Email Reply	Good (morning/afternoon). Thank you for sending through the (attached/below) correspondence. I can acknowledge receipt of this email, and that this information will be forwarded to the appropriate staff member. If you have any queries, please feel free to contact (07) 3156 9400 or email back to MetrosouthACATReferrals@health.qld.gov.au Kind Regards,
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Appointment Booked:

Face to Face Assessment Booked	Phone call with (<i>client/carer</i>) to schedule a Face-to-Face appointment for a Comprehensive Aged Care assessment. Requested information to be made available to the assessor at the appointment. HVRA completed. Appointment time confirmed for (<i>dd/mm/yyyy</i>) at (<i>time</i>) with (<i>clinician</i>). (<i>Name</i>) (<i>Relation</i>) will be present. (<i>Language</i>) interpreter booked. XX admin
Phone Assessment Booked	Phone call with (<i>client/carer</i>) to schedule a Phone appointment for a Comprehensive Aged Care assessment. Requested information to be made available to the assessor at the appointment. Please call (<i>Client/carer</i>) on Mobile (<i>Phone number</i>). Appointment time confirmed for (<i>dd/mm/yyyy</i>) at (<i>time</i>) with (<i>clinician</i>). (<i>Name</i>) (<i>Relation</i>) will be present. XX admin
Clinic Assessment Booked	Phone call with (<i>client/carer</i>) to schedule a Face-to-Face clinic appointment for a Comprehensive Aged Care assessment at (<i>Name of Clinic</i>). Requested information to be made available to the assessor at the appointment. Appointment time confirmed for (<i>dd/mm/yyyy</i>) at (<i>time</i>) with (<i>clinician</i>). Confirmation of appointment letter sent/emailed to (<i>client/carer</i>). (<i>Name</i>) (<i>Relation</i>) will be present at <i>ax</i> . (<i>Language</i>) interpreter booked. XX admin
Home Support Assessment Booked	Phone call with (<i>client/carer</i>) to schedule a Face-to-Face appointment for a Home Support Aged Care assessment. Requested information to be made available to the assessor at the appointment. HVRA completed. Appointment time confirmed for (<i>dd/mm/yyyy</i>) at (<i>time</i>) with (<i>clinician</i>). (<i>Name</i>) (<i>Relation</i>) will be present. (<i>Language</i>) interpreter booked. XX admin
Appointment Confirmation Email	Dear (<i>client/carer</i>), This email is to confirm your (<i>Appointment type</i>) Aged Care assessment with (<i>clinician</i>) from the Aged Care Assessment Service (ACA). Time: Date: For the Assessment, please have with you: Medicare Card

Text Messages:

Wait Time Inquiry	Thank you for your inquiry regarding the wait time for an Aged Care Assessment. You or your representative will be contacted by our administrative team when your referral comes up in the waitlist. Regards, ACA Admin PLEASE DO NOT REPLY
Missed Call	This is Metro South Aged Care Assessment Service trying to contact you regarding an appointment for a (Home Support Comprehensive) Aged Care Assessment. Please call us on 07 3156 (<i>Extension</i>). Regards, QLD Health PLEASE DO NOT REPLY
Appointment Confirmation	Dear (<i>client/carer</i>), This message is to confirm your (<i>Appointment type</i>) Aged Care assessment with (<i>clinician</i>) from the Aged Care Assessment Service (ACA). Time: Date: For the Assessment, please have with you: - Medicare Card - Pension/ Health care or DVA card - List of current medications - GP health summary and other current medical reports Cancellation: For any cancellation or rescheduling enquiries, please phone (ACA) Aged Care Assessment Team on 3156 9400. Kind regards,

7 Local Emergency Response Procedures

Please see the attached guide on the next page.

WORK INSTRUCTION

Medical Emergency – ‘Code Blue Response’ Eight Mile Plains Community Health Centre

WI 2016/102
Version No. 2.2

PURPOSE

This Workplace Instruction (WI) provides guidelines regarding management of a medical emergency at Eight Mile Plains Community Health Centre including the process to deliver appropriate care.

HUMAN RIGHTS ACT 2019

Metro South Hospital and Health Service is committed to respecting, protecting and promoting human rights. Under the Human Rights Act 2019, Metro South Hospital and Health Service has an obligation to act and make decisions in a way that is compatible with human rights and when making a decision, to give proper consideration to human rights. When deciding about a Code Blue Medical Emergency Response, decision makers must comply with that obligation.

SCOPE

This work instruction applies to all employees (permanent, temporary or casual) working within the Eight Mile Plains Community Health Centre who may assist in a medical emergency. This includes operational, technical, administrative or clinical staff, providing this care is within their respective competency and scope of practice. Staff who have completed annual Basic Life Support (BLS) training will be expected to recognise acute deterioration and deliver cardio-pulmonary resuscitation (CPR).

The WI provides information to ensure that staff understands the duty that they have in the provision of assistance in a timely manner.

Contents of this WI include:

1. **Local emergency response** – Defines the responses by first responders to a medical emergency within the centre and adjoining grounds. There is NO Rapid Response team in this facility so Queensland Ambulance Service (QAS) must be called on 000 (remember to dial 0 to get an outside line) in ALL instances of a Medical Emergency.
2. **Equipment** – Lists the equipment available and its location within the centre.

This WI should be read in conjunction with the following guidelines:

- [GL 2019/56 Emergency /Duress Call System Eight Mile Plains Community Health Centre](#)
- [PR 2021-249 Community and Oral Health Code Blue - Medical Emergency](#)

[PR 2021-249 Community and Oral Health Code Blue - Medical Emergency](#) procedure outlines in detail:

- Criteria that define a Medical Emergency
- Basic Life Support for Non-COVID cases
- Basic Life Support for Suspected or Confirmed COVID-19 cases

Together these documents describe how staff will recognise and respond to a medical emergency.

WORK INSTRUCTION

1. LOCAL EMERGENCY RESPONSE

When a person is unresponsive and breathing abnormally or is deemed to have signs of acute deterioration, the Code Blue call is initiated; and the local area response system put in place. The process to follow will depend on which area of the building the 'medical emergency' occurs. It is each individual staff member's responsibility to make themselves aware of the emergency buttons in use in the area they work.

In areas where the Emergency/ Duress call buttons are installed (Ground Floor ONLY):

Use either Yellow or Green or Black if that is the closest button to the emergency event.

- When pressed an alarm will sound
- Annunciator screens will indicate STAFF ASSIST or NURSE ASSIST or DURESS
- The location of the emergency will scroll across the annunciator screen
- ALL available staff will proceed to the location to assist
- First responder to ascertain nature of emergency
- If CODE BLUE- Follow Code Blue - [PR 2021-249 Community and Oral Health Code Blue - Medical Emergency](#) procedure and commence DRSABCD

Other staff may:

- Ring 666 from nearest phone to alert reception re 'Code Blue Medical emergency' **and**;
- Phone 0-000 request Queensland Ambulance Service (QAS)
 - QAS may need to speak directly to staff member at site of incident to relay information
 - State type of medical emergency
 - State street address of Eight Mile Plains Community Health Centre and where in the building: Eight Mile Plains Community Health Centre, 51 McKechnie Drive, Eight Mile Plains, QLD 4113
 - Begin documentation on [Medical Event / Resuscitation form](#).

Where there is NO emergency/Duress call system

In all other areas within Eight Mile Plains Community Health Centre where there is no access to the emergency call buttons, staff will need to follow the DRSABCD protocols (see flow chart page 5) to alert a medical emergency situation.

This may include but is not limited to:

1. Shout out for help (if appropriate to do so)
2. Ring 666 from nearest phone to alert reception of 'Code Blue Medical Emergency' **and**
3. Phone 0-000 requesting Queensland Ambulance Service

Role of First Responders in a Medical Emergency

Where the first responder is a clinician, they should take a lead role in managing the medical emergency including: assess the person, call for help, ensuring the safety of themselves and the person. Before commencing resuscitation consider:

- Is it likely the person has COVID-19?
- Is there public health advice to wear a mask in the local area?

If 'YES' to either criteria, first responders **MUST** don full airborne Personal Protective Equipment (PPE) including N95/P2 mask prior to commencing cardiac compressions. Manual ventilation should not be performed.

When the first responder is not a clinician, they may be required to provide initial supportive care until clinical assistance or QAS arrives.

Refer to [PR 2021-249 Community and Oral Health Code Blue - Medical Emergency](#) which outlines the appropriate actions for management of both suspected, probable or confirmed COVID-19 cases and non-COVID 19 cases.

Administration staff receiving call re: 'Medical Emergency' when alarm call system has NOT been activated.

- Go to 'Fire panel' located outside the main entrance; and use Public Address system to notify 'Code Blue - Medical Emergency' and alert staff in building that additional assistance is required
- Announce location of emergency
- May also announce 'no further assistance required', if notified by clinical lead

2. EQUIPMENT

The following equipment is available at the Eight Mile Plains Centre Community Health Centre:

Semi-Automatic External Defibrillator (SAED)	Air Viva & Oxygen tubing	Oxygen cylinder	Medical Event / Resuscitation Record Forms
Defibrillation Pads x 2 sets	Non-sterile Gloves	Razors x 2	Scissors x 1
Long sleeve impermeable gowns x 3	Face shields x 3	Flat surgical mask x 2	N95 / P2 masks x 3
Hudson Mask x 2	Goggles x 1	Laminated instructions for correct process for Donning / Doffing PPE	



Image 1: Philips FR2+ defibrillator



Image 2: Location of Emergency Equipment

- A Philips FR2+ defibrillator (image 1) and associated equipment in an 'Emergency Box' (image 2) is located in the gym on the Ground Floor.
- Brisbane South Chronic Disease team will be responsible for the maintenance, re-stocking and checking of the emergency equipment. The emergency equipment is to be checked each day Monday-Friday excluding Public Holidays.
- For any clinical waste, utilise 'spill kit' located adjacent to the emergency warden board on the ground floor and dispose of in appropriate clinical waste bin.

- Following the use of equipment in a medical emergency, the team responsible for checking equipment must be notified.
- Replacement of equipment is to be requested by emailing Community and Oral Health Director of Nursing MSHCOH-DON@health.qld.gov.au

CHECKLIST RETENTION

The Emergency Equipment checklists (attachment 2) are to be tabled at the facility management meeting of each site and are to be retained / stored with facility management meeting documentation.

AUDIT

The authors of this work instruction will arrange for annual audits to be undertaken to review compliance in the daily checking of the medical emergency equipment and provide recommendations for improvement.

SUPPORTING DOCUMENTS

[Princess Alexandra Hospital Procedure Manual 'Code Blue Medical Emergency'](#)

[GL 2019/56 Emergency /Duress Call System Eight Mile Plains Community Health Centre](#)

ANZCOR Guidelines (Australian and New Zealand Committee on Resuscitation) via [Australian Resuscitation Council \(ARC\)](#)

[PR 2021-249 Community and Oral Health Code Blue - Medical Emergency](#)

[National Covid-19 Clinical Evidence Flowcharts 6 & 7 and 10 CPR of Adults with Covid-19 in Healthcare settings Australian Resuscitation Council \(ARC\)](#)

[The NSQHS Standards | Australian Commission on Safety and Quality in Health Care](#)

[PR2020/203 Management of Patients with Known or Suspected Coronavirus \(COVID-19\) in the Community](#)

RELATED DOCUMENTS

[Medical Event/Resuscitation form](#)

Attachment 1: Adult Basic Life Support for Community & Oral Health Services during the COVID-19 Pandemic flowchart

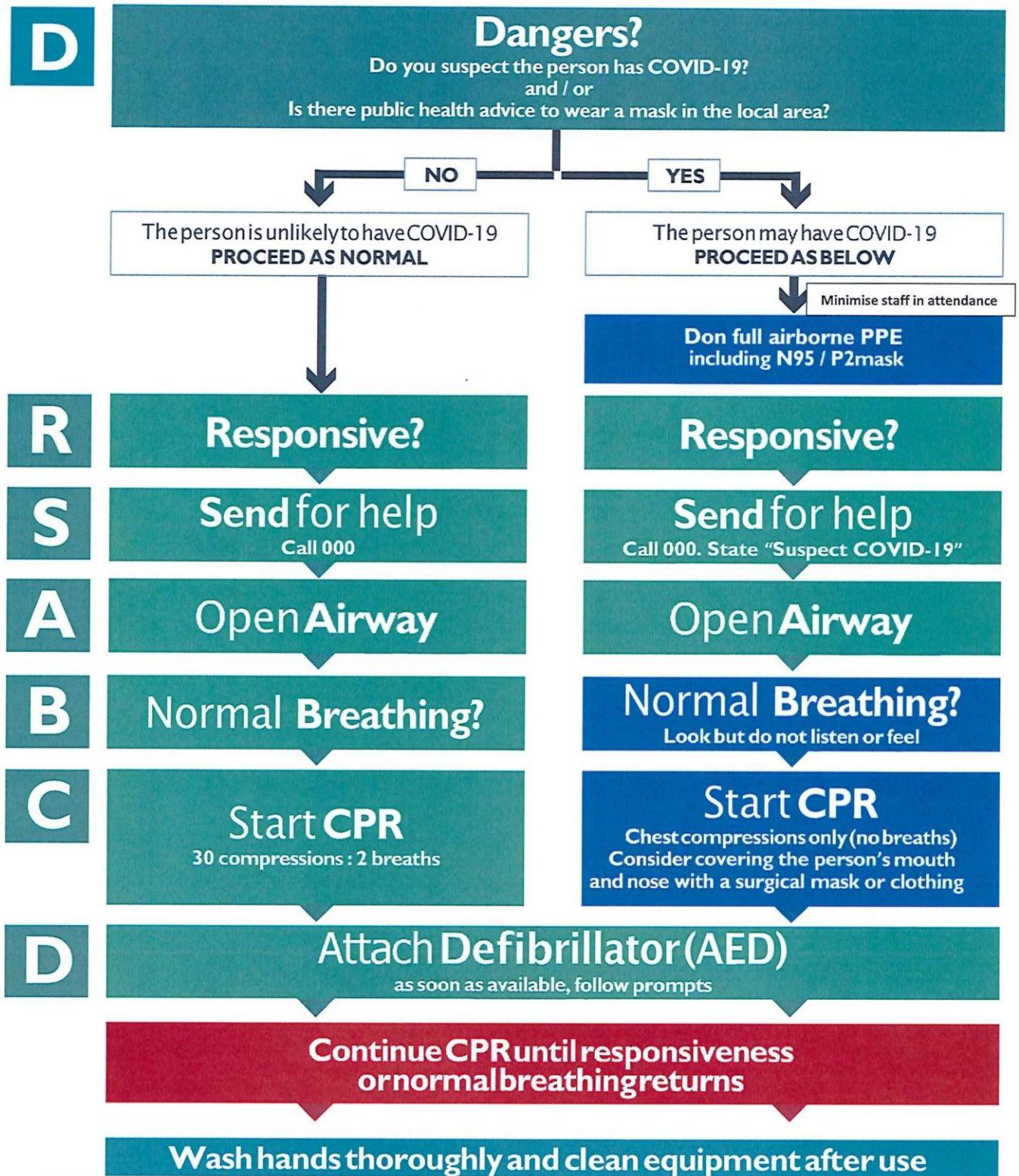
Attachment 2: Emergency Equipment Checklist – Eight Mile Plains Community Health Centre

Attachment 3: Emergency Equipment Checking – Eight Mile Plains Community Health Centre

DEFINITIONS

ARC	Australian Resuscitation Council
BLS	Basic Life Support
CHC	Community Health Centre
CPR	Cardio-Pulmonary Resuscitation
DRSABCD	Danger, Response, Send for help, Airway, Breathing, Compressions, Defibrillator
QAS	Queensland Ambulance Service
SAED	Semi automated external defibrillator
WI	Work Instruction

ADULT BASIC LIFE SUPPORT FOR COMMUNITY AND ORAL HEALTH SERVICES DURING THE COVID-19 PANDEMIC



Adapted by NPDU Community Feb 2021

based on National COVID-19 Clinical Evidence Taskforce Flowchart 10 "CPR for Adults in the Community During the COVID-19 Pandemic" - Oct 22 2020
and Flowchart 6 "CPR in Healthcare Settings Dec 2 2020"

WORK INSTRUCTION DETAILS

Work Instruction Number

WI 2016/102 Version 2.2

Work Instruction Name

Medical Emergency – Code Blue Response
Eight Mile Plains Community Health Centre

Policy Reference

PL 2015/0041
Metro South Clinical Governance Policy

Supersedes

WI 2016/102
Version 1.0

Procedure Author

CN Quality Chronic Disease
Nurse Educator Community Based Services

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Approving Officer

Director of Nursing Community & Oral Health

Approving Date

June 2021

Effective From

31 July 2017

Date of Last Review

June 2021

Date of Next Review

June 2024

Local Code Black Protocol

Aged Care Assessment Team

Eight Mile Plains Community Health Building

To ensure appropriate local response to a Code Black Event

Roles and Responsibilities

ROLE	LOCATION	ACTION
Emergency Officer (EO) or appropriate person		Take charge of situation If threat is outside unit: • Direct a staff member to contact Queensland Police Service (QPS) on 000 • Direct a staff member to call Switch on Ext 666 to advise switch of threat and to enable switch to inform other building occupants of threat • Direct a staff member to ensure door to Level 1 office space (D1.11) remains closed and no one exits • Direct unit occupants to assemble at the rear of the office area out of sight of entrance • Await further instruction from QPS • Evacuate area to a point of safety via emergency exit Fire Safety exits if necessary and safe to do so If threat is inside unit: • Contact QPS or direct a staff member to do so • Direct staff to exit the unit via unit entrance (D1.11) and emergency fire exits if required
Staff and visitors		• Follow instructions of EO and QPS • Remain out of sight

SITE MAP – to identify staff 'SAFE RETREAT' or 'SAFE AREA' doors and Emergency Officer locations

